

DIPLOMA IN SOCIAL SERVICE

Social Service Programme Implementation

SSC-PMT-4024-1.1

Reflective Practice

Assignment Cover Sheet

Name of Candidate TAN VIE MING	Host Organisation
Name of Assessor LUCY	Contact Details 98891383
Name of IA Supervisor	Date of Submission

CANDIDATE'S DECLARATION

a. I, hereby declare that all the entries in this journal are my own effort and do not involve plagiarism or works of other people whose services I have engaged.

b. I have not allowed, and will not allow, anyone to copy any of my Written Assignments with the intention of passing it off as his/her own works.

Signature: _____ Date: _____

**Attach this Assignment Cover sheet to your RP journal.*

Introduction to Reflective Practice Assignment

A reflective journal is a personal record of your learning experiences. It is a tool which enables you to record and reflect upon your observations and responses to situations, which can then be used later to explore and analyse ways of thinking and being in contexts. Journals, although generally written, can also contain images, drawings and other types of reference materials.

This Reflective Journal uses the basic framework for self-reflection which is *description, reflect and application*. For this Reflective Practice assignment, you are required to complete this reflective journal of experiences gathered during the industrial attachment.

Instructions

Pre-Attachment

1. You will be assigned to a Host Organisation (HO) for your industrial attachment (IA). A HO supervisor will be appointed to mentor you.
2. If you have any questions regarding the entries in the RPJ, email your assessor for clarifications.
3. HMTA office will contact you to book the date for the Final Assessment before the IA. The date and timeslot selected by you is final. Any changes may incur an administrative fee.

During Attachment

4. A week prior to your IA, craft some guiding questions related to the TSC listed in the journal. You may gather in groups of 2 or 3 to design the questions. These questions will help you to target the areas you would like to learn during the attachment.
5. Preferable to make an appointment with your IA-Supervisor within the first week of attachment to gather the information related to the TSC. In addition, reflect on your work roles (if any) performed each workday which are related to any of the TSC in the RP journal.
6. Document the information gathered from your IA-supervisor, your

personal observations or the work roles you performed every day in Column 2 of the RPJ for the respective modules.

7. The writing of the RPJ is an individual and personal assignment.

8. The RPJ should be used to explore situations from a personal perspective, but generally within the context of learning from your experiences. Write your personal reflections in Column 3 on the information gathered in Column 2 of the RPJ. To guide you in writing your entry, you may find the following sample questions useful:

- *Is there a framework?*
(a) If no, why not? What would you have recommended to the HO?
(b) If yes, does it serve its intended purpose? Is there a set of principles to guide the organisation in managing the framework? What would you have recommended to the HO?
- *What were the incidents? What were the outcomes? Were these incidents preventable? If yes, what could be done differently to prevent the incidents?*

9. You are advised to make your entries after each workday and at your own pace.

Post-Attachment

10. Email the RPJ to your assessor 5 days after the end of the industrial attachment and cc to the Course Co-ordinator.

During RP Presentation

11. You have 30 mins to present your entries for each module to the assessor. The following sequence is recommended:

- Provide a brief overview of the information gathered.
- Share your thoughts and feelings on what you have learnt.
- Highlight your key learning points from your experience.

12. Assessor may pose a few questions to clarify your entries.

13. Candidate must demonstrate competency in both the RPJ and the RP presentation, in order to be deemed competent for this RP.

Quality of the Journal Entries

Entries should:

- be clear and reflect your ability to think critically and objectively.
- be in complete sentences. You are allowed to number or bullet your entries for easy reading.
- demonstrate you have acquired the respective technical skills and competencies of this module.
- journal all interactions, experiences, thoughts and relevant information which are deemed relevant
- not breach client confidentiality
- avoid using names when referring to people at the workplace

Social Service Programme Implementation (SSC-PMT-4024-1.1)

LOs & TSC	Design 2 guiding Questions per row	Summarise the Information	Reflect on the information and pen your observations, thoughts, and experiences in relation to the respective LOs and TSC.
LO1 Learners will be able to adapt the best practices and strategies to develop and implement a fundraising programme to suit the needs of clients or programme objectives. <u>TSC</u> A1 Adapt programmes and services to suit needs of client or programme objectives			

LOs & TSC	Design 2 guiding Questions per row	Summarise the Information	Reflect on the information and pen your observations, thoughts, and experiences in relation to the respective LOs and TSC.
K1 Best practices and strategies to adapt programmes			
LO2 Learners will be able to build and utilise networks of community stakeholders and social service organisations to extend support and participation in programmes. <u>TSC</u> A4 Utilise networks within the community and social service organisations to			

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extend support and participation in programmes. K4 Methods to build community stakeholder support for programmes			
LO3 Learners will be able to review a fundraising programme to identify areas for improvement and to address the causes for client disengagement <u>TSC</u> A2 Develop strategies to address			

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causes for client disengagement			

LO1

A1 Adapt programmes and services to suit needs of client or programme objectives

To Develop Appropriate Social Service Programmes for the Elderly Aged 65 and Above

Project Summary

The project targets seniors aged 65 and above living in 2-room rental units at Blk 310 Clementi Ave 4, a mature estate with high-density older housing. It combines a structured Community Needs Assessment with a pilot intervention, program design, survey tools to move from data to early, preventive support and a community-centered narrative.

Objectives

1. Identify challenges and needs — map health, social, emotional and functional risks among target seniors.
2. Develop a realistic action plan — co-design solutions with local assets, using a collaborative delivery model.

Location and Key Touchpoints

- Core site: Blk 310 Clementi Ave 4
- Immediate partners: St Luke's Eldercare and Active Ageing Centre (SLEC), SASCO@ West Coast (704 West Coast Road), Clementi Meadows Residents' Network (306 Clementi Ave 4), Clementi Community Club (220 Clementi Ave 4), Social Service Office, Family Service Centre.
- System alignment: Healthier SG and Age Well SG, where Active Ageing Centres are being transformed from targeted social points into population-wide health-social hubs aiming to reach 80% of seniors by 2025

Community Needs Assessment — Six-Step Framework

1. Profile the Community

Bukit Merah is a mature, high-density planning area with older HDB blocks and a concentration of seniors in 1-room units, indicating need for age-friendly outreach and in-place support.

2. Define Scope

Target 100 seniors in May 2026. Delivery by 10 trained volunteers working in pairs for safety, consistency and PDPA-compliant data handling.

3. Identify Assets

Active Ageing Centres, Senior Activity Centres, FSCs, SSOs, AIC Silver Generation Office, community hubs, faith groups, hawkers, clinics, and local sponsors.

4. Connect Stakeholders

Social workers, medical social workers, AIC care coordinators, AAC centre managers, NCSS, grassroots leaders, caregivers and small businesses — aligned to Singapore's "Many Helping Hands" approach where CDCs work with corporate and community partners to complement national schemes. The approach prioritizes individual responsibility supported by family and community.

5. Collect Data

Face-to-face interviews and structured observations focused on four domains: emotional, cognitive, social, and physical well-being. Tools: short-form needs screener, social isolation scale, ADL checklist, and asset map.

6. Analyse and Prioritise

Triangulate findings to identify immediate risks (e.g., medication non-adherence, falls, loneliness), available protective factors, and gaps requiring early intervention.

K1: Best Practices to Adapt Programs and Services

The selected program is a pilot workshop for high-risk seniors and their caregivers. It is built on two principles:

1. **Asset-Based Community Development (ABCD)** — start with strengths, peer skills and neighbourhood spaces
2. **Many Helping Hands (MHH)** — coordinate across health and social sectors for shared accountability

Design features, adapted from national learning:

- Co-create content with seniors. Duke-NUS research on AAC implementation found older adults form stronger social connections when encouraged to co-create programmes.
- Meet seniors where they are. Seniors prioritise comfort and familiarity, preferring hawker centres and void decks over unfamiliar venues, and programmes should maintain low barriers and embed connections in existing routines.
- Keep sessions brief and digestible. Deliver a 3-hour modular workshop, using small focus groups of 6 to 8 to encourage sharing among caregivers and reduce cognitive load.
- Integrate health-social screening. Include basic vital-sign checks, falls risk, and social prescribing referrals, consistent with AAC 2.0 innovations such as peer-led programmes and extended hours.
- Use trust-based engagement. National commentary recommends strengthening community partnerships and investing in trust-based engagement to address fragmented coordination and limited community voice.
- Apply robust, simple evaluation. Administer a short post-event survey, track attendance, referrals made, and follow-up uptake. Embed evaluation

frameworks informed by implementation science, as proposed for AAC 2.0 scale-up.

- Manage resources transparently. Maintain a detailed line-item budget, volunteer roster, and a coordination timeline from needs assessment through program closure, with clear roles for AAC, RN and FSC.

This approach keeps the intervention realistic, collaborative, and grounded in Bukit Merah's local assets, while aligning with Healthier SG and Age Well SG priorities for preventive, community-rooted aging in place.

LO2 Learners will be able to build and utilise networks of community stakeholders and social service organisations to extend support and participation in programmes.

TSC

A4 Utilise networks within the community and social service organisations to extend support and participation in programmes.

Agency for Integrated Care (AIC) serves as the central “care aggregator”, coordinating support for aged home (specifically nursing homes and community care facilities) through several key pillars:

Referral and Care

AIC acts as the main gateway for seniors needing long-term care.

Care Placement

- They manage referrals from hospitals and the community, ensuring seniors are placed in appropriate nursing homes based on their medical and social needs.
- Single Point of Contact: Through the AIC hotline and AIC Links, they assist families in navigating the various residential care options available.

Financial Assistance and Subsidies

AIC administers government schemes that make aged homes more affordable for Singaporeans.

Means-tested Subsidies

- They manage the framework that provides up to 75% subsidy for residential long-term care services (like nursing homes) for eligible lower-to middle income families.

Grant Administration

AIC oversees various disability and caregiving grants (eg. CareShield Life, Elderfund) that can help cover the monthly costs of staying in a care facility.

Capability Building and Training

AIC provides significant support to the workforce to ensure high-quality care within these homes

- **Manpower Development:** They offer study awards and scholarships to build a pipeline of nursing and therapy talent for the community care sector.

Staff Training

AIC provides training subsidies and programs for staff in aged homes to upgrade their skills in areas like dementia care, palliative care, and wound management.

Digitalization and Productivity

AIC helps aged homes modernize their operations through the Community Care Digital Transformation Plan

- **Funding for Technology:** Grants such as the Productivity and Digitalisation Grant help home adopt clinical management systems, electronic health records, and assistive technology (like fall detection sensors)
- **Process Improvement:** They provide guidance on business process re-engineering to help homes run more efficiently with limited manpower

K4 Methods to build community stakeholder support for programmes

Engage Grassroots Organisations (GROs):

Partner with People's Association (PA) entities like Residents' Network (RN). They hold the "ground pulse" and can provide physical venues (void decks, CCs) and direct access to residents.

Cross-Sector Partnerships (Corporate and Religious)

Corporate Social Responsibility (CSR)

Engage local businesses for "Skilled-based Volunteerism" for example, a tech firm in an industrial estate can provide digital literacy training for a nearby Senior Activity Centre or Active Ageing Centre.

Inter-Racial and Religious Confidence Circles

Given Singapore's cultural diversity, engaging religious organizations is vital. They often have dedicated welfare wings and a ready pool of committed donors and volunteers.

Effective Use of Digital Platforms

- [Giving.sg](#): Utilize this centralized national portal for fundraising and volunteer recruitment to build credibility
 - Community Apps: Leverage the OneService app or estate-specific Telegram groups to share success stories and upcoming activities, keeping the programme top-of-mind for residents
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LO3 Learners will be able to review a fundraising programme to identify areas for improvement and to address the causes for client disengagement

Develop strategies to **address causes for client disengagement**

Developing effective strategies to address client disengagement requires a multifaceted approach that tackles both internal programme design and external life factors. In the Singapore social service context, this involves aligning with national frameworks like **Healthier SG** and **Age Well SG** while addressing the specific needs of seniors and their caregivers.

The following strategies are designed to address the primary causes of disengagement:

1. Managing Expectations and Programme Outcomes

Disengagement often occurs when there is a mismatch between what was promised and what is delivered.

- **Deliver on Promised Outcomes**: Ensure the programme consistently provides the specific benefits or results communicated at the outset to maintain trust.
- **Clear Communication and Goal Setting**: Set realistic, transparent goals from the beginning so clients have a clear understanding of what to expect.
- **Emphasize Incremental Progress**: Use milestones to highlight small successes, which helps maintain participant motivation and a sense of achievement.
- **Feedback-Driven Adjustments**: Regularly review programme outcomes against client expectations and adjust offerings based on participant feedback.

2. Addressing External Barriers and Caregiver Burnout

External stressors, particularly within the family unit, are significant drivers of senior disengagement.

- **Support for Caregivers:** Address caregiver burnout, depression, or family conflict directly, as these factors often lead seniors to withdraw to avoid being a "burden".
- **Respite and Resource Linkage:** Connect caregivers to **Agency for Integrated Care (AIC)** resources, such as respite care, support groups, and financial assistance, to alleviate the pressure on the home environment.
- **Befriending Outreach:** Deploy befriending volunteers to maintain a "soft" connection with seniors who have disengaged, providing social support without the immediate pressure of programme participation.

3. Enhancing Client Agency and Personalization

Clients are more likely to stay engaged when they feel a sense of ownership and relevance.

- **Co-Creation of Content:** Involve seniors in the design of programmes. Research indicates that older adults form stronger social ties when they are encouraged to co-create activities rather than just receive them.
- **Platform for "Voice":** Provide meaningful platforms for seniors to share their experiences (e.g., through storytelling, theater, or mentoring) which empowers them with a sense of purpose and "generativity".
- **Respect for Preferences:** Tailor engagement to individual comfort levels, such as offering one-to-one interactions before transitioning to larger group settings.
- **Cultural and Linguistic Inclusion:** Use native languages or dialects and create culturally relevant content to ensure seniors feel comfortable and understood.

4. Proactive Community Outreach and Re-engagement

For clients who have already disengaged, a proactive and systematic approach is necessary.

- **Senior Profiling:** Conduct profiling exercises (similar to those by the **People's Association**) to identify "hidden" or vulnerable seniors who are not captured in existing databases.
- **Neighbourly Care:** Strengthen community networks where active seniors look out for their peers, facilitating re-engagement through informal social visits and errands.
- **Gradual Re-introduction:** Encourage family members to accompany seniors to activities initially to ease anxiety and build the confidence needed for long-term participation.
- **Digital Connection:** Use simple technology (e.g., video calls or messaging apps) focused on maintaining social connections rather than technical mastery, particularly for those with limited mobility.